

Parametric Survival Modeling

METIMMOX-1 Economic Evaluation

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Table of contents

1	Overview	2
2	Population Characteristics including Biomarker Prevalences	2
3	Model Selection and Comparison	2
3.1	Analysis Specifications	2
3.2	Data Considerations	3
3.3	Model Validation and Goodness of Fit	3
4	Model Comparison	3
5	Model Coefficients	4
5.1	Overall Survival (OS) Model Coefficients	4
5.2	Progression-Free Survival (PFS) Model Coefficients	4
6	Population Survival Curves	5
6.1	Model Validation: Comparison with Kaplan-Meier Estimates	5
7	Biomarker-Specific Results	7
7.1	CRP Biomarker Strategy	8
7.2	TLR Biomarker Strategy	9
7.3	TMB_BRAF Biomarker Strategy	10
8	Strategy Comparison	11
9	Summary	13

1 Overview

This report presents parametric survival models fitted for the METIMMOX-1 economic evaluation. The models compare different biomarker strategies for overall survival (OS) and progression-free survival (PFS) outcomes. This analysis serves as the foundation for the cost-effectiveness evaluation by quantifying treatment effects across biomarker-defined patient subgroups.

2 Population Characteristics including Biomarker Prevalences

Continuous variables:

- Age (mean): 63.98 years

Categorical variables (most frequent category used):

- Sex: 1 (0=female, 1=male)
- CRP: 0 (0=negative, 1=positive)
- TLR: 1 (0=negative, 1=positive)
- TMB_BRAF: 0 (0=negative, 1=positive)

Biomarker prevalences in population:

- CRP positive: 33.8 %
- TLR positive: 64.7 %
- TMB_BRAF positive: 44.9 %

Data completeness:

- Complete cases used: 65 of 74 observations (87.8 %)

3 Model Selection and Comparison

3.1 Analysis Specifications

- **Distributions tested:**
 1. Exponential
 2. Weibull
 3. Weibull PH
 4. Gompertz
 5. Log-logistic
 6. Log-normal
 7. Generalized Gamma
 8. Gamma
 9. Generalized F Distribution
- **Covariate structure:** Full models with age and sex adjustment, treatment effects, and treatment-biomarker interactions:
 1. Age (continuous, years)
 2. Sex (categorical, 0=female, 1=male)
 3. Treatment assignment (Control arm vs Experimental arm)
 4. CRP biomarker status (0=negative, 1=positive)
 5. TLR biomarker status (0=negative, 1=positive)
 6. TMB_BRAF biomarker status (0=negative, 1=positive)
 7. Treatment $\times$ CRP interaction
 8. Treatment $\times$ TLR interaction

9. Treatment $\times$ TMB_BRAF interaction

Note: Models without age and sex adjustment were also considered but are not used in the primary analysis.

3.2 Data Considerations

- **Complete case analysis:** Only patients with complete covariate data included
- **Reference patient:** Modal categories for categorical variables, mean for continuous variables
- **Time horizon:** Models fitted to observed follow-up period with extrapolation capability
- **Biomarker prevalences:** Taken from the METIMMOX-1 trial and may or may not reflect real world settings

3.3 Model Validation and Goodness of Fit

- **Selection criterion:** AIC is more appropriate here because:
 - BIC penalizes model complexity more heavily (uses $\log(n)$ vs 2 for AIC)
 - For prediction purposes, we want the model that best balances fit and complexity
 - BIC tends to select overly simple models that might miss important biomarker interactions
 - AIC is better for predictive modeling while BIC is better for identifying the “true” model
- **Validation:** Visual comparison with Kaplan-Meier curves and clinical plausibility assessment

4 Model Comparison

Table 1: Overall Survival (OS) model comparison using AIC and BIC

Structure	Distribution	AIC	BIC	Delta AIC	Rank	Selected
Full Model	gamma	622.56	646.47	0.00	1.0	***
Full Model	weibull	623.44	647.36	0.88	2.5	
Full Model	weibullph	623.44	647.36	0.88	2.5	
Full Model	llogis	623.68	647.60	1.12	4.0	
Full Model	lognormal	624.09	648.01	1.53	5.0	
Full Model	gengamma	624.56	650.65	2.00	6.0	
Full Model	genf	626.56	654.82	4.00	7.0	
Full Model	gompertz	633.09	657.01	10.53	8.0	
Full Model	exponential	654.14	675.88	31.58	9.0	

Table 2: Progression-Free Survival (PFS) model comparison using AIC and BIC

Structure	Distribution	AIC	BIC	Delta AIC	Rank	Selected
Full Model	gamma	453.43	477.35	0.00	1.0	***
Full Model	lognormal	453.46	477.38	0.03	2.0	
Full Model	weibull	454.19	478.11	0.76	3.5	
Full Model	weibullph	454.19	478.11	0.76	3.5	
Full Model	llogis	455.16	479.08	1.73	5.0	
Full Model	gengamma	455.28	481.37	1.85	6.0	
Full Model	genf	457.28	485.55	3.85	7.0	
Full Model	gompertz	467.57	491.49	14.14	8.0	
Full Model	exponential	483.52	505.26	30.09	9.0	

5 Model Coefficients

5.1 Overall Survival (OS) Model Coefficients

Selected model:

- Distribution: gamma
- Covariate adjustment: Age and sex adjusted

Table 3: Regression coefficients for the selected OS parametric survival model

Parameter	Estimate	SE
shape	1.2275	0.1820
rate	-2.8975	0.6157
Age (per year)	0.0015	0.0082
Sex (male vs female)	0.1454	0.1482
RxExperimental arm	-0.2309	0.2852
RxControl arm:crp1	-0.3570	0.3162
RxExperimental arm:crp1	-0.3395	0.2265
RxControl arm:tlr1	-0.8039	0.2509
RxExperimental arm:tlr1	-0.4109	0.2129
RxControl arm:tmb_braf1	-0.0866	0.2210
RxExperimental arm:tmb_braf1	-0.1859	0.2072

5.2 Progression-Free Survival (PFS) Model Coefficients

Selected model:

- Distribution: gamma
- Covariate adjustment: Age and sex adjusted

Table 4: Regression coefficients for the selected PFS parametric survival model

Parameter	Estimate	SE
shape	1.2426	0.1901
rate	-1.8647	0.6321
Age (per year)	-0.0009	0.0084
Sex (male vs female)	0.1321	0.1653
RxExperimental arm	0.4218	0.3073
RxControl arm:crp1	0.0654	0.4253
RxExperimental arm:crp1	-0.5754	0.2412
RxControl arm:tlr1	-0.9764	0.2718
RxExperimental arm:tlr1	-1.1914	0.2133
RxControl arm:tmb_braf1	-0.1055	0.2537
RxExperimental arm:tmb_braf1	-0.4891	0.2325

6 Population Survival Curves

The following figures compare empirical Kaplan-Meier estimates with predicted survival curves from the selected parametric models. “Population” refers to different patient groups: biomarker-positive patients, biomarker-negative patients, or the entire population. For the control strategy, population curves represent all patients. For biomarker-guided strategies, population-level predictions are weighted by the prevalence of biomarker-positive and biomarker-negative patients in the sample.

6.1 Model Validation: Comparison with Kaplan-Meier Estimates

These figures show how well each tested parametric distribution fits the observed data for the entire study population.

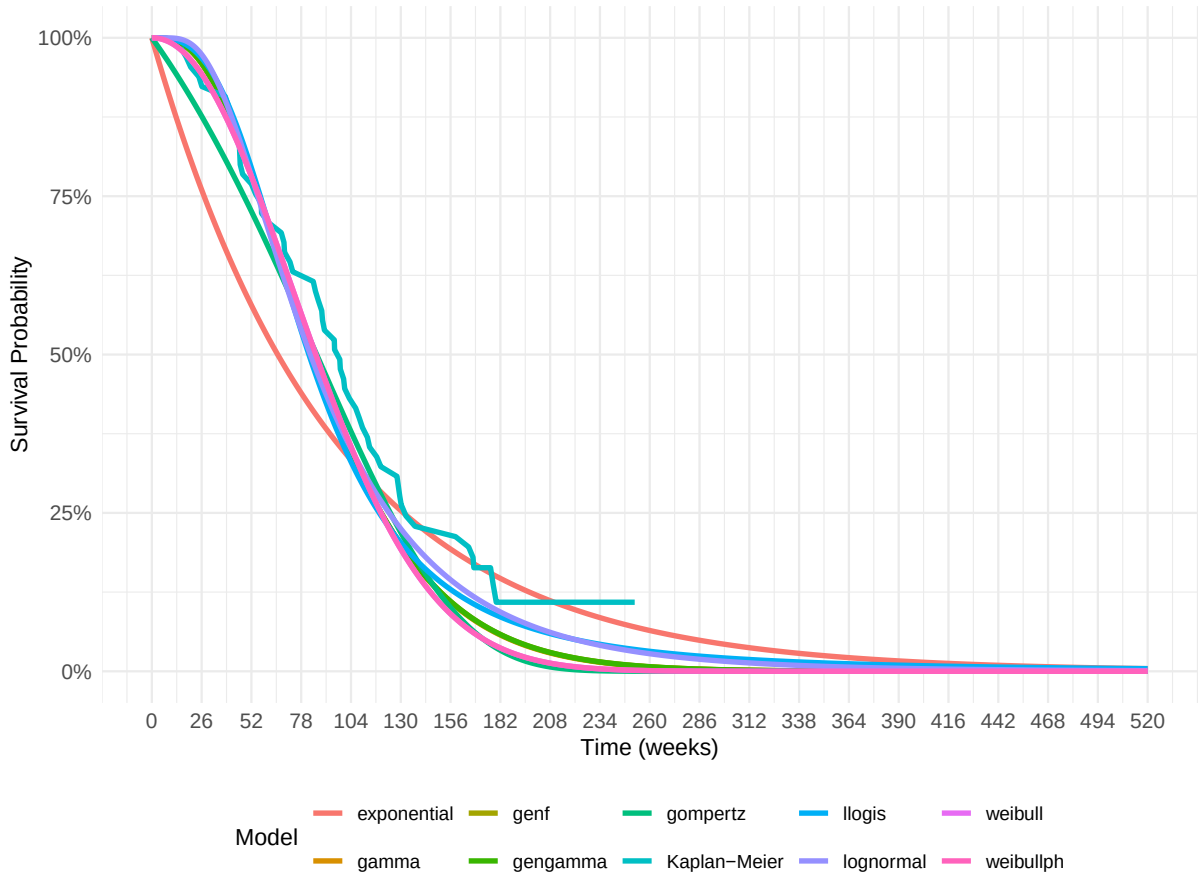


Figure 1: Overall survival curves for age- and sex-adjusted models compared to Kaplan-Meier estimates. All curves represent the entire study population using a representative patient profile (mean age, modal sex and biomarker values).

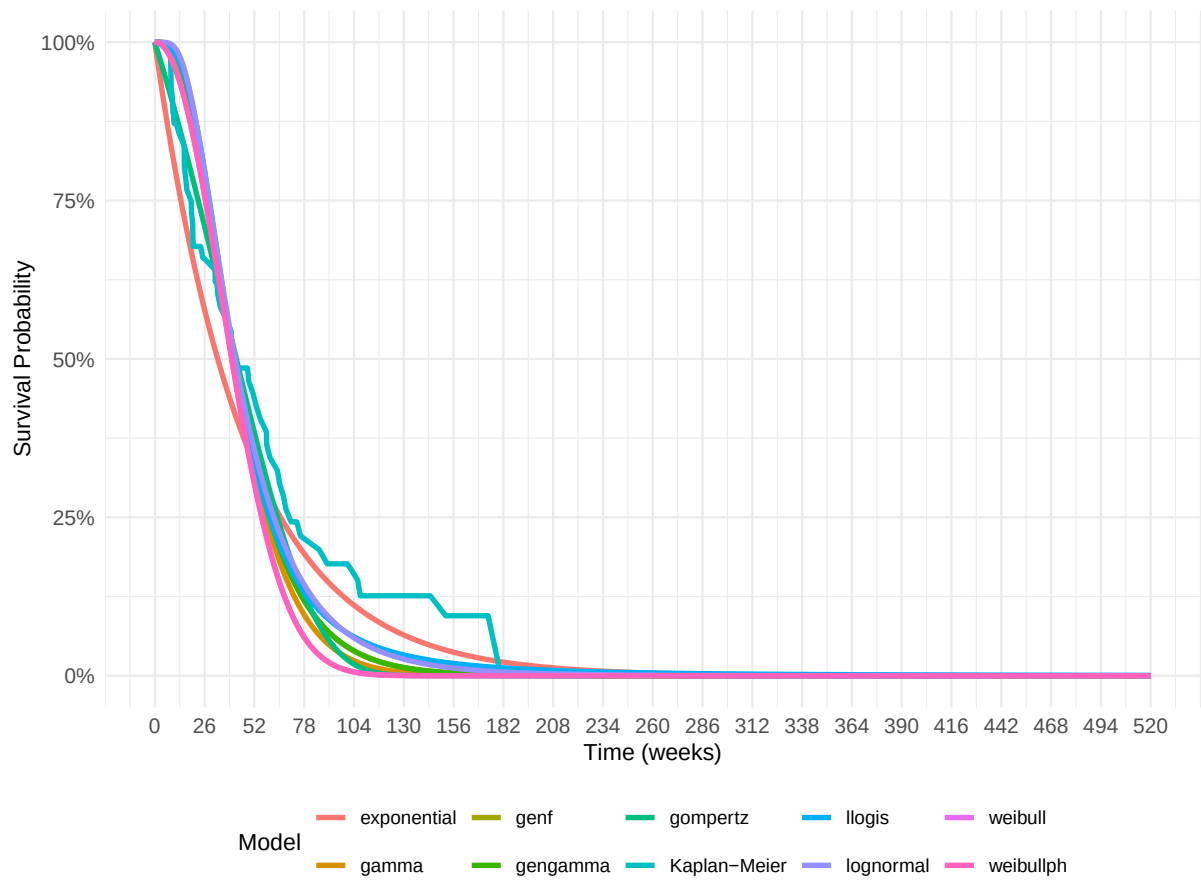


Figure 2: Progression-free survival curves for age- and sex-adjusted models compared to Kaplan-Meier estimates. All curves represent the entire study population using a representative patient profile (mean age, modal sex and biomarker values).

7 Biomarker-Specific Results

The following figures compare survival outcomes between biomarker-positive and biomarker-negative patient subgroups for each biomarker strategy. These comparisons inform the potential value of biomarker-guided treatment selection. Kaplan-Meier estimates are shown for biomarker-positive patients receiving experimental treatment and biomarker-negative patients receiving control treatment, alongside parametric model predictions.

7.1 CRP Biomarker Strategy

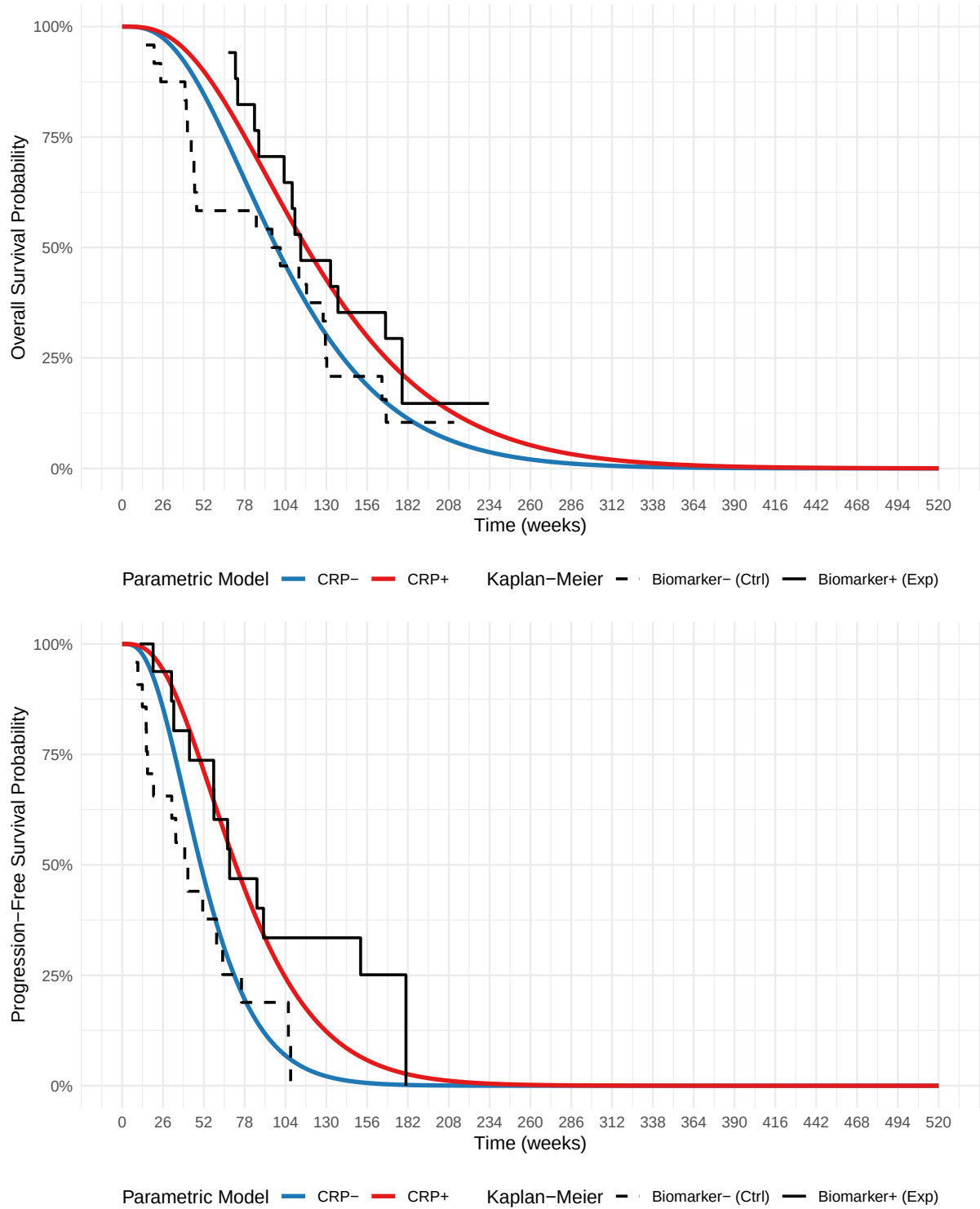


Figure 3: Overall survival and progression-free survival for CRP biomarker-positive versus biomarker-negative patients. Colored curves show predicted survival from the selected parametric model. Black curves show Kaplan-Meier estimates for the treated (biomarker-positive, experimental arm, solid line) and untreated (biomarker-negative, control arm, dashed line) subgroups. Differences between curves indicate the predictive value of CRP status for treatment benefit.

7.2 TLR Biomarker Strategy

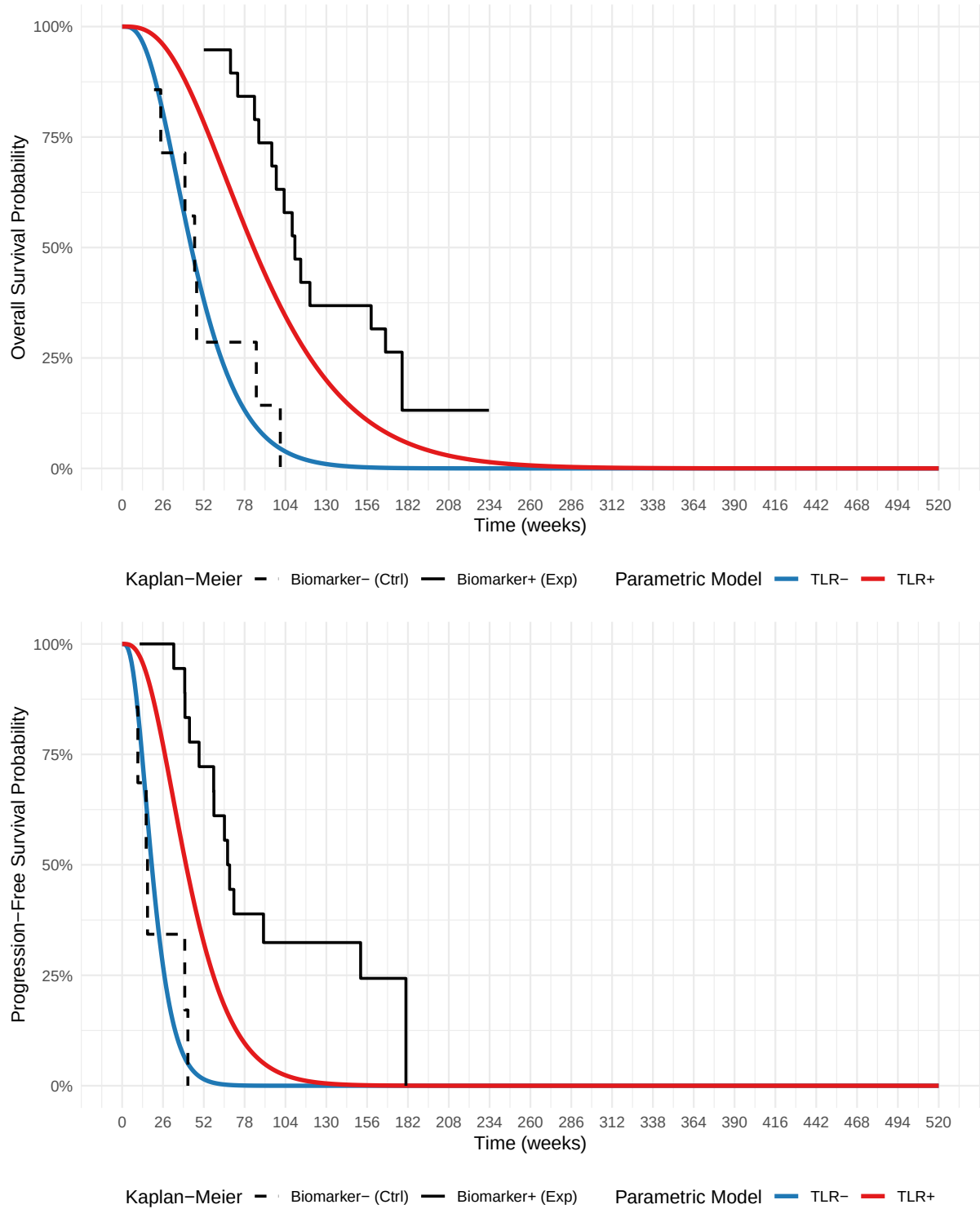


Figure 4: Overall survival and progression-free survival for TLR (target lesion reduction) biomarker-positive versus biomarker-negative patients. Colored curves show predicted survival from the selected parametric model. Black curves show Kaplan-Meier estimates for the treated (biomarker-positive, experimental arm, solid line) and untreated (biomarker-negative, control arm, dashed line) subgroups. Differences between curves indicate the predictive value of TLR status for treatment benefit.

7.3 TMB_BRAF Biomarker Strategy

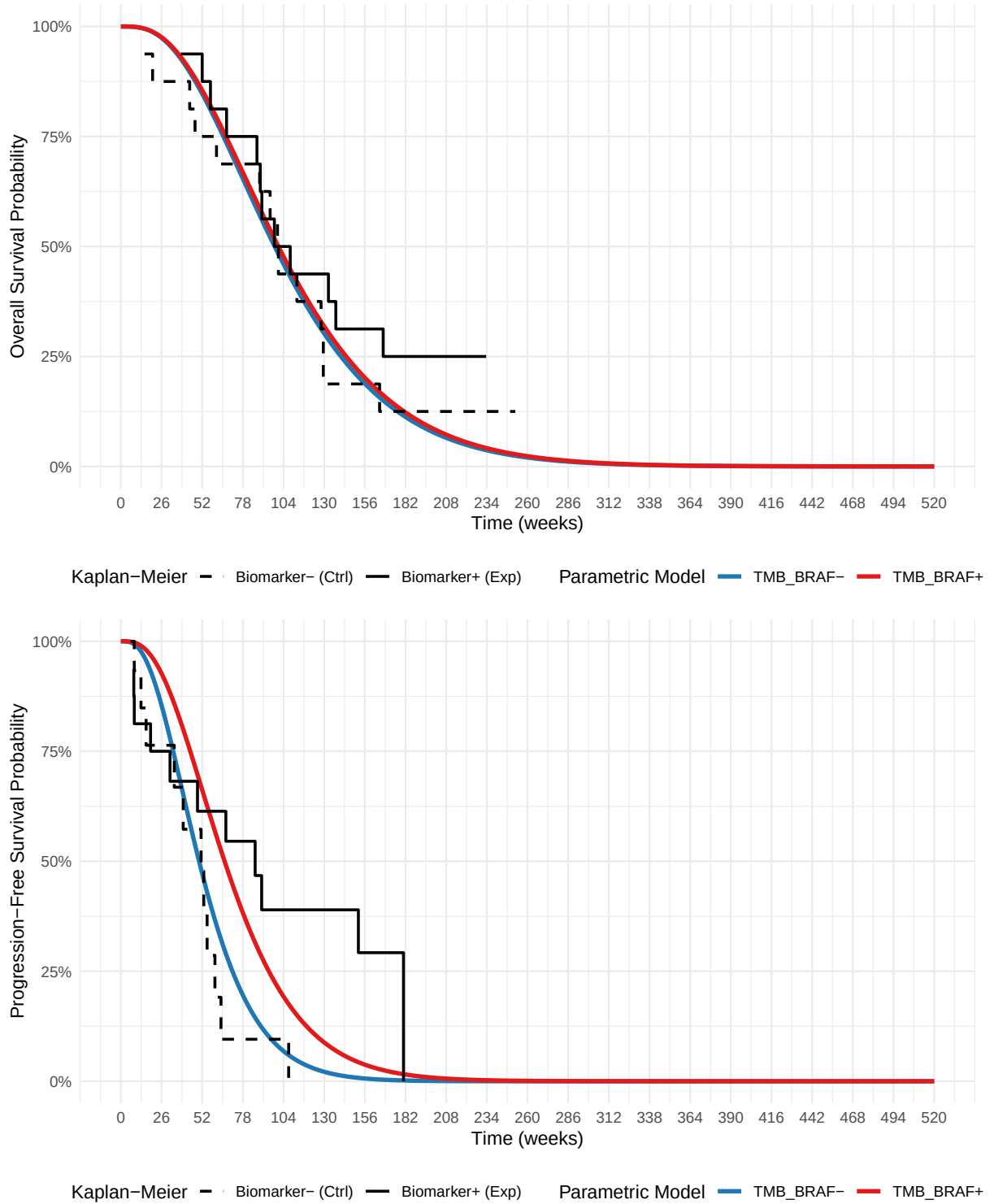


Figure 5: Overall survival and progression-free survival for TMB_BRAF biomarker-positive versus biomarker-negative patients. Colored curves show predicted survival from the selected parametric model. Black curves show Kaplan-Meier estimates for the treated (biomarker-positive, experimental arm, solid line) and untreated (biomarker-negative, control arm, dashed line) subgroups. Differences between curves indicate the predictive value of TMB_BRAF status for treatment benefit.

8 Strategy Comparison

The following figures compare all three biomarker-guided treatment strategies against the control (standard of care) strategy. These population-level predictions are weighted by biomarker prevalence to reflect expected outcomes in an unselected population.

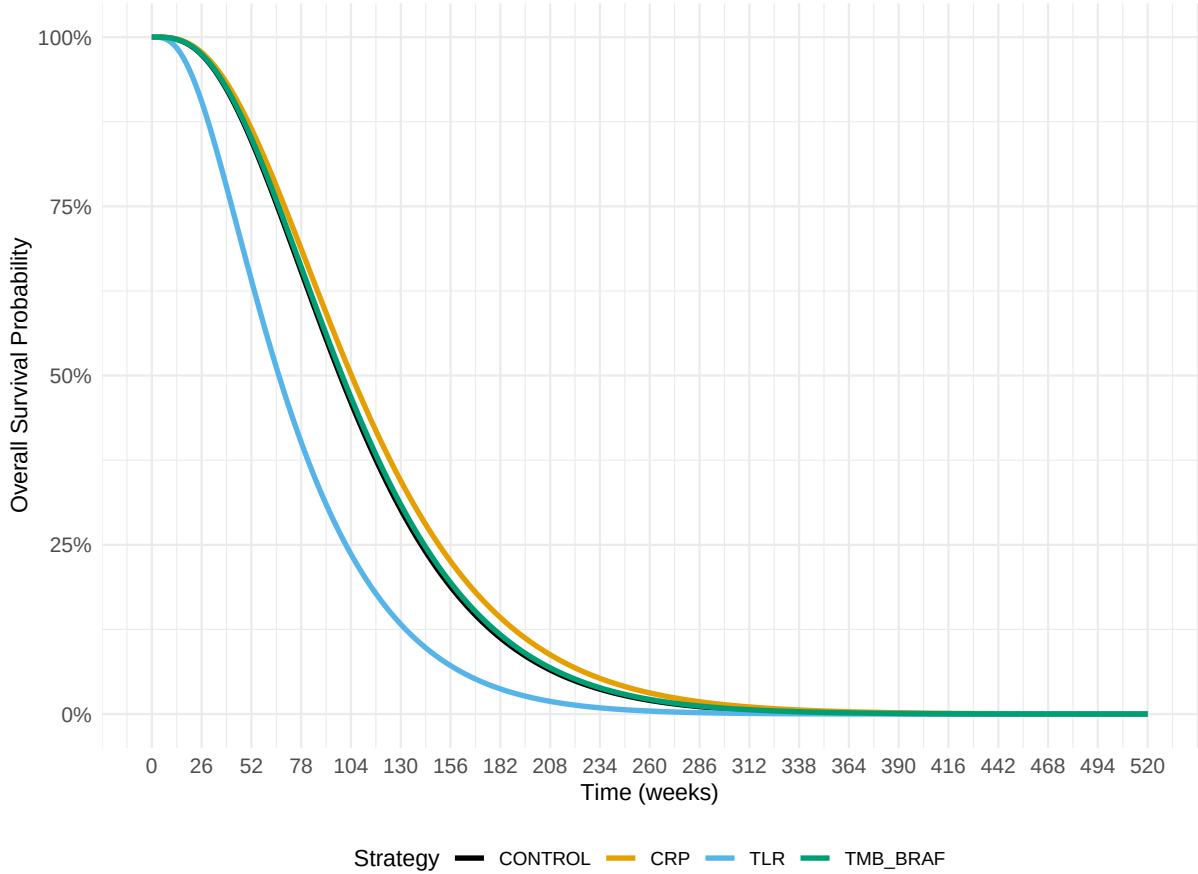


Figure 6: Overall survival comparison across all treatment strategies. Each curve represents the population-weighted expected OS for each strategy: Control (standard of care for all patients), CRP-guided treatment allocation, TLR-guided treatment allocation, and TMB_BRAF-guided treatment allocation. Differences between strategies reflect both the predictive accuracy of each biomarker and its prevalence in the population.

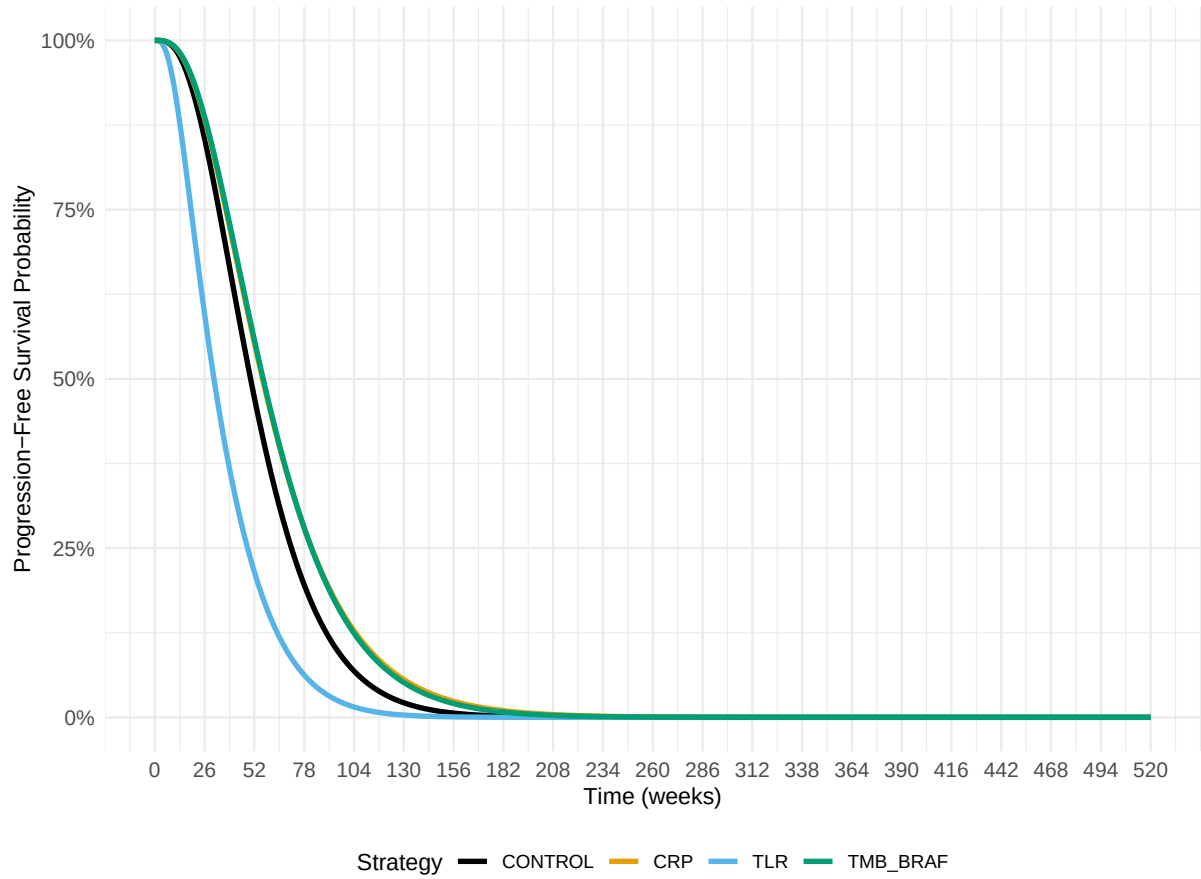


Figure 7: Progression-free survival comparison across all treatment strategies. Each curve represents the population-weighted expected PFS for each strategy: Control (standard of care for all patients), CRP-guided treatment allocation, TLR-guided treatment allocation, and TMB_BRAF-guided treatment allocation. Differences between strategies reflect both the predictive accuracy of each biomarker and its prevalence in the population.

9 Summary

- Parametric survival models were fitted for both overall survival (OS) and progression-free survival (PFS) outcomes
- Model selection was based on Akaike Information Criterion (AIC) to optimize predictive performance
- Age- and sex-adjusted models with treatment-biomarker interactions were used in the primary analysis
- Biomarker-specific survival patterns were characterized for CRP, TLR (target lesion reduction), and TMB_BRAF strategies
- Visual validation against Kaplan-Meier estimates confirms good model fit across all tested distributions
- Population-level predictions account for biomarker prevalence to reflect expected outcomes in clinical practice
- Strategy comparison shows relative performance of biomarker-guided treatment allocation approaches

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